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Name: منى ابو الوفا خليفه عثمان National ID: 27303310100581	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

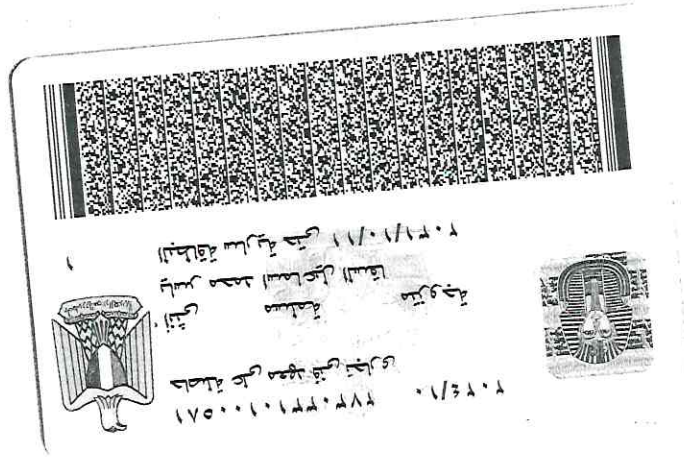
نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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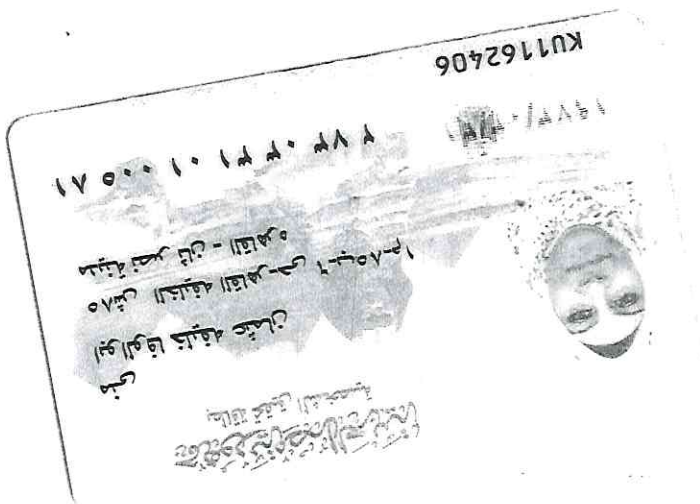
وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Contrast-enhanced CT Chest & abdomen and pelvis
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



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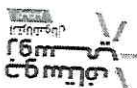
Patient's Name: Mona Abu Al-Wafa Khalifa Uthman

Date: 2026-07-20

Referred by: Prof. Dr. no doctor

Hospital:

Goustave Roussey International Egypt



Clinical summary: Positive family history (mother), complaining of bilateral breast lump.
Findings:

Right breast:

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- Upper breast central middle third, irregular high density mass lesion with indistinct margin, along with surrounding distortion.
- Thickened skin and coarsened trabeculae.
- Irregular distortion demonstrated upper breast, 11-1 o'clock reaching the nipple, 4 mm deep to the skin, measuring 4.5 x 4.5 x 3.5 cm.
- 11 o'clock posterior third small hypoechoic heterogeneous irregular small mass measuring 16 x 14 mm.
- Thickened skin reaching 6 mm and coarsened trabeculae.
- 11 o'clock (more than three) pathological level 1, 2, & 3 axillary LNs are noted, along with one supra-clavicular node measuring 7 x 7 mm, one of level 1 pathological nodes showed extra-vascular features.

Left breast:

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- Global central asymmetry with marked surrounding distortion and overlying regional coarse heterogeneous suspicious micro-calcifications.
- Irregular distortion demonstrated 11-2 o'clock, middle and anterior third, irregular heterogeneous non mass area of altered breast parenchyma reaching the nipple mounting to mass formation measuring about 6 x 4 x 2 cm.
- 11 o'clock (more than three) pathological level 1, 2, & 3 axillary and supra-clavicular LNs are noted.

Conclusion:

- Right breast (11-2 o'clock) highly suspicious advanced process with pathological axilla & supra-clavicular lymph nodes.
- Left breast (11-2 o'clock): highly suspicious process with pathological axillary & supra-clavicular lymph nodes.

Best Regards, DR. Amal Hegazi, M.Sc.

Patient Name	منى ابو الوفا خليفه	Case No.	345853
Sex/Age	Female 53 Y	Patient No.	25502
Registered Date	03/8/2026 03:08:58 PM	Referred By Dr.	Interventional Radiology
Validation Date	16/9/2026 11:24:28 AM	Branch	Gustave Roussy
Reporting Date	26/9/2026 11:12:27 AM	Pathology No.	2440/26

Pathology Report

Clinical Data: Bilateral breast masses. BIRADS 4c.

Nature of Specimen:

1- Right breast 11-1 O'clock mass, Tru-cut biopsy, collected on 03/08/2026 at 2:20 pm, Cold ischemia time: 20 seconds.

2- Left breast 11-2 O'clock mass, Tru-cut biopsy, collected on 03/08/2026 at 2:20 pm, Cold ischemia time: 20 seconds.

Gross Examination:

Two containers were received, designated as follow:

1- Right breast mass: Three grey, white tissue cores, the largest one measured 1.0 cm long, totally submitted.

2- Left breast mass: Fragmented grey, white tissue cores, ranging in size between 0.2 & 1.0 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

1- Right breast mass:

- Histologic type: Invasive duct carcinoma.

- Histologic grade: 2 (Differentiation 2, pleomorphism 3, mitosis 2).

- DCIS: Absent.

- Angioinvasion: Absent.

- Tumor infiltrating lymphocytes (TILs): High.

* IPT (All controls showed appropriate reactivity):

- ER: No nuclear reaction in tumor cells (score 0/8).

- PR: Weak nuclear reaction in <1% of tumor cells (score 2/8).

- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).

- SISH: Not amplified.

(Average Her2 copy No.: (2.4), Her2/CEP17 ratio: (1.6)).

- Ki67%: 40%.

2- Left breast mass:

- Histologic type: Invasive duct carcinoma with squamous metaplastic component.

- Histologic grade: 2 (Differentiation 2, pleomorphism 3, mitosis 2).

- DCIS: Intermediate grade DCIS (Solid pattern).

- Angioinvasion: Absent.



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*** IPT (All controls showed appropriate reactivity):**

- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.
- (Average Her2 copy No.: (3.1), Her2/CEP17 ratio: (2.69)).
- Ki67%: 25%.

Diagnosis:

Right breast mass (1) & Left breast mass (2), Tru-cut biopsies:

1- Right breast mass:

- Invasive duct carcinoma, Grade 2.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 2/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified. Group V.
- (Average Her2 copy No.: (2.4), Her2/CEP17 ratio: (1.6)).
- Ki67%: 40%.

2- Left breast mass:

- Invasive duct carcinoma, Grade 2.

- Intermediate grade ductal carcinoma in situ (Solid pattern).

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified. Group II.
- (Average Her2 copy No.: (3.1), Her2/CEP17 ratio: (2.69)).
- Ki67%: 25%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Patient Name	منى ابو الوفا خليفه	Case No.	345853
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Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells. ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Prof. Dr. Maha Guinei.

Dr. Walaa Ibrahim Ghonain, MD.



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